

# HIGHLIGHTS OF PRESCRIBING INFORMATION: ATORVASTATIN CALCIUM TABLETS

## 1 INDICATIONS AND USAGE

Atorvastatin calcium is indicated:

- To reduce the risk of myocardial infarction, stroke, revascularization procedures, and angina in patients with multiple risk factors for coronary heart disease (CHD) or type 2 diabetes.
- As an adjunct to diet to reduce elevated total cholesterol, LDL-C, apolipoprotein B, and triglycerides in patients with primary hyperlipidemia and mixed dyslipidemia.
- For primary dysbetalipoproteinemia and homozygous familial hypercholesterolemia.

## 2 DOSAGE AND ADMINISTRATION

- Dose range: 10 mg to 80 mg orally once daily, taken at any time of day, with or without food.
- Recommended starting dose: 10 mg or 20 mg once daily. For patients requiring large LDL-C reduction (> 45%), start at 40 mg once daily.
- Assess lipid levels within 2 to 4 weeks after initiation and adjust dosage accordingly.

## 4 CONTRAINDICATIONS

- Acute liver failure or decompensated cirrhosis.
- Hypersensitivity to atorvastatin or any excipients in the tablets.

## 5 WARNINGS AND PRECAUTIONS

- Myopathy and Rhabdomyolysis: Rare cases of rhabdomyolysis with acute renal failure secondary to myoglobinuria have been reported. Risk is increased in elderly patients ( $\geq 65$ ), patients with uncontrolled hypothyroidism or renal impairment, and when coadministered with certain interacting medications. Advise patients to promptly report unexplained muscle pain, tenderness, or weakness.
- Liver Enzyme Elevations: Persistent elevations in hepatic transaminases (ALT/AST > 3 times upper limit of normal) can occur. Perform liver enzyme tests before initiating therapy and as clinically indicated thereafter.
- Increases in HbA1c and Fasting Glucose: Increases in blood sugar levels have been reported with statins.

## 6 ADVERSE REACTIONS

Most common adverse reactions (incidence  $\geq 2\%$ ):

- Nasopharyngitis, arthralgia, diarrhea, pain in extremity, urinary tract infection, dyspepsia, nausea, musculoskeletal pain, muscle spasms, and insomnia.

## 7 DRUG INTERACTIONS

- Strong CYP3A4 Inhibitors (e.g., Clarithromycin, Itraconazole, Ketoconazole, HIV/HCV protease inhibitors): Substantially increase atorvastatin plasma concentrations and risk of myopathy. Avoid coadministration or limit atorvastatin dose to 20 mg.
- Cyclosporine or Gemfibrozil: Concomitant use increases risk of rhabdomyolysis. Avoid coadministration.
- Grapefruit Juice: Ingestion of large quantities (> 1.2 liters daily) increases atorvastatin exposure; avoid large quantities.
- Digoxin: Steady-state plasma digoxin concentrations increase by approximately 20%; monitor digoxin levels.